NCCP 00280 · clinician draft

Cisplatin + Etoposide

Which treatment this guide covers This guide covers Cisplatin + Etoposide chemotherapy for extensive-stage small-cell lung cancer. Immunotherapy combinations and limited-stage treatment with radiotherapy use separate regimens. If an additional medicine or radiotherapy is prescribed, obtain its own plan and consent information.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cisplatin + Etoposide · standard three-day course

Active medicines: cisplatin + etoposide

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	NCIS high-risk reference course for Day 1 platinum Reconcile the full multi-day course; the special extension after five-day cisplatin is not automatically applied to this three-day etoposide course.
Ondansetron	16 mg orally Day 1 (high-risk reference)	Before Day 1 treatment NCIS low-risk etoposide-day reference is ondansetron 8 mg orally once daily or a corticosteroid. Days 2–3 prevention must be reconciled with the existing dexamethasone course; do not duplicate agents automatically. Adjust for QT/ individual risk.
Dexamethasone	12 mg orally Day 1; 8 mg orally daily Days 2–4 (high-risk reference)	A component of the individual Days 1–5 plan These are the NCIS high-risk row doses. Pharmacy must reconcile overlapping delayed-platinum and etoposide-day prevention, and specify the Day 5 plan; do not invent a uniform steroid extension.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days / rescue instructions set by prescriber High-risk NCIS row has no end day or PRN qualifier. The low-risk row uses PRN. Specify role/duration and check suitability; avoid unreviewed duplication.
Olanzapine	5 mg orally Day 1 and daily Days 2–4 (high-risk reference)	Part of the reviewed multi-day antiemetic plan Review sedation and metabolic effects. Concurrent metoclopramide increases extrapyramidal risk. Consultant may consider 10 mg; do not alter automatically.

Additional phase-specific support

- NCIS V8 p 5: provide antiemetics appropriate to the risk of SACT on each treatment day and for two days after; for standard Days 1–3 treatment, review coverage through Day 5.
- Day 1 high-risk platinum and Days 2–3 low-risk etoposide are different exposures. Table 1 alone does not give one universal three-day platinum/etoposide prescription.
- Do not invent extra steroid days or extend aprepitant using the separate five-day cisplatin exception.
- Confirm one written steroid/antiemetic course, QT risk, interactions and rescue medicines.
- IV and oral etoposide alternatives require matching pharmacy and patient instructions.
- Source suggested hydration: etoposide in 1 L 0.9% NaCl over 1 hour may serve as prehydration; after cisplatin,10 mmolMgSO4+20 mmolKCl in 1 L 0.9% NaCl over 2 hours, adapted to local policy and patient fluid/electrolyte status.
- Mannitol may be used per local policy; routine furosemide is not recommended without fluid overload.
- If cisplatin is fractionated over three days, each day’s risk, hydration and antiemetic course need separate consultant/pharmacy reconciliation.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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Sources checked 2026-10-26 · SACTCheck v0.84.23 · Regimen-level drafts; prescriber and oncology-pharmacy verification required